GAMSA · Gender-Affirming Medicines South Australia

Blood test guide: Testosterone short-acting injection

Testosterone short-acting injection (enantate, mixed esters) · gamsa.au/monitoring

Key points Take the blood sample midway between injections. Aim: within the male range on the lab report. Haematocrit below 0.50 L/L (50%).

Applies to me	Test	When	Aim	Talk to your prescriber if
☐	**Testosterone level (short-acting injection: enantate or mixed esters)**	Midway between injections, or as a pair of tests (peak and trough) if your prescriber asks	Within the male reference range on the lab report. The 10 to 20 nmol/L trough figure does not apply to a midway sample Other guidance (Endocrine Society 2017): midway, about 14 to 24 nmol/L (400 to 700 ng/dL)	It is outside the male reference range
☐	**Full blood count (haemoglobin and haematocrit)**	Before starting, then at each review. Checked more closely if you take an SGLT-2 inhibitor (such as empagliflozin or dapagliflozin)	Haematocrit below 0.50 L/L (50%), read against the male range	It is 0.50 L/L (50%) or more: smoking, sleep apnoea and lung or kidney problems are checked, and the interval or dose may be changed, or a gel or cream used instead
☐	**Liver function, lipids and glucose**	Before starting, then at each review or as your prescriber decides	Within the lab's range	Your prescriber flags a change
☐	**Pregnancy test**	Before starting, if a pregnancy is possible. Testosterone is not contraception	—	You could be pregnant: testosterone can harm a developing baby, so tell your prescriber straight away

A guide to what to expect, not a schedule to follow on your own. Your prescriber sets the timing.

Reading your results

- Lab reports mark results as high or low using the sex on your Medicare or lab record. On hormones, that range may not fit your body. In the first year your prescriber may ask the lab for both the male and the female ranges.
- A single flagged result is a reason to talk with your prescriber, not to change a dose on your own.
- Units: Australian labs report oestradiol in pmol/L, testosterone in nmol/L, and haematocrit as a fraction (0.50 L/L is the same as 50%).

Things that can make a result look wrong

- A sample taken at a different time after the dose from the one your prescriber asked for.
- Haemoglobin and haematocrit: read against the range for your affirmed gender once hormones have had time to work (about 3 months on testosterone, about 6 months on oestradiol). On testosterone, the male range is used.
- Kidney function (eGFR) is worked out from your creatinine, age and recorded sex, so the change over time tells more than one result.
- Iron studies: the range for your affirmed gender, unless you are pregnant or have periods (then the female range is used).

Sources: AusPATH 2026 pp21, 22, 35 (Table 7), 36, 42, 55; WPATH SOC8 Appendix C; Equinox guide 2020 pp6, 7; Primoteston Depot and Sustanon 250 PI s4.2, s4.4; 2019 position statement Contraception, Monitoring; Reandron 1000 PI s4.4. Full references and detail for clinicians: gamsa.au/monitoring.

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